



Altered Mental Status

Assessment

Pediatric Pearls:

- Use pediatric dosing of medications or electrical therapy for a pediatric patient < 37 kg and as defined by the PEDIA Tape.
- Use volume control device (IV Burette) for Dextrose Infusions.
- Upper limit BGL is 200

Signs & Symptoms:

- Decrease mental status
- Change in baseline mental status.
- Bizarre behavior
- Hypoglycemia (cool, diaphoretic skin)
- Hyperglycemia (warm & dry skin, fruity breath, Kussmaul respirations, signs of dehydration)

Differential:

- Hypoxia
- Brain trauma
- CNS (Stroke, Tumor, Seizure, Infection)
- Cardiac (MI, CHF)
- Infection
- Thyroid (hyper or hypo)
- Shock (septic, metabolic, traumatic)
- Toxicological / Carbon Monoxide / Cyanide
- Acidosis / Alkalosis
- Heat Stroke or Hypothermia
- Electrolyte abnormality

Clinical Management Options

P	P	P	P	P	P	• Oxygen, target SpO₂ 92 – 96% • BGL Assessment, if BGL < 50 and intact gag reflex then Oral Glucose & ◦ Consider turning off insulin pump if present • Basic Airway Management as needed
L	L	L	L	L	L	• Assess for a Stroke and follow Stroke COG accordingly • Monitor ETCO₂
1	2	3	4	5	6	• Vascular access as appropriate for patient condition • If BGL < 50 then Dextrose Infusion titrated to patient condition and response • If BGL < 50 and no IV access, then Glucagon • If BGL > 300 in adults or > 200 in pediatrics or signs of dehydration, then infusion of isotonic crystalloid
						• Cardiac monitor and 12 ECG • Advanced Airway Management as needed

Consult Online Medical Control As Needed

Pearls:

- Refer to drug formulary charts for all medication dosing for both adults and pediatric patients.
- Be aware of AMS as presenting sign of an environmental toxin or Haz-Mat exposure and protect personal safety.
- It is safer to assume hypoglycemia than hyperglycemia if doubt exists. Recheck blood glucose after Dextrose or Glucagon.
- Do not let alcohol confuse your clinical practice as alcoholics frequently develop hypoglycemia.
- Hyperglycemia is treated with fluids since these patients are volume depleted.
- Patients on oral hypoglycemics or long-acting insulin are at risk for repeat episodes of hypoglycemia; monitor closely and encourage transport.
- If a hypoglycemic patient has returned to baseline and wishes to refuse care, ensure that the patient eats and that there is someone to observe them for repeat hypoglycemic episodes.
- Blood samples for performing glucose analysis should be obtained through a finger-stick (heel for infants). Venous blood samples may produce artificially high blood glucose values and should be avoided.